



POLICY

Inpatient Mental Health Services Admission to Discharge

Scope (Staff):	All NMHS Mental Health (MH) Staff
Scope (Area):	All NMHS MH Areas

Contents

1. Aim	1
2. Background	1
3. Risk	3
4. Definitions	3
5. Principles	4
6. Admission procedures.....	5
7. Discharge procedures	6
8. Clinical Handover	7
9. Documentation	8
10. Compliance and Evaluation	8

1. Aim

To effectively identify admission and discharge processes to facilitate ease of access to inpatient mental health services for consumers, families and carers and provide timely and planned discharge to appropriate services as required in accordance with individual needs and preferences.

To ensure that there are processes in place to facilitate the early identification, early access to care and treatment and safe transition back to the community through collaborative care planning taking into account the additional requirements of people with identified vulnerabilities, and those at increased risk of harm to self and others.

2. Background

This policy is consistent with the following requirements of the [Mental Health Act 2014](#), [the WA Department of Health Safety Planning for Mental Health Consumers Policy and Procedure](#) and and the [Chief Psychiatrist Clinicians' Practice Guide to the Mental Health Act 2014](#).

Clinical activities relating to the Admission Readmission Discharge and Transfer of patients in inpatient mental health facilities are in accordance with the requirements of the [MHA 2014](#) and the WA DoH [Triage to Discharge Mental Health Framework for State-wide Standardized Clinical Documentation \(SSCD\)](#).

The [Triage to Discharge Mental Health Framework for State-wide Standardized Clinical Documentation \(SSCD\)](#) describes the requirements for service to complete clinical documentation pertaining to mental health triage, admission, treatment and discharge to and from mental health service and timelines that must be adhered to.

[The National Standards for Mental Health Services](#) outlines responsibilities for services in the provision of patient centred mental health practices.

[DoH Code of Conduct Policy \(MP 0124/19\)](#) and the [DoH Integrity Governance Policy \(MP 0114/19\)](#) require staff to adhere to professional standards and practice in accordance with the DoH policies, service policy documents and in accordance with the values of the organisation; Care, Respect, Integrity, Trust and Innovation (NMHS Values).

[The Chief Psychiatrist Standards for Clinical Care WA](#) identifies the following requirements for mental health services:

- To facilitate equitable access and improved mental health outcomes for Aboriginal people with mental illness, and their carer's, families and communities, by delivering mental health services that take into account the cultural and social diversity of Aboriginal people with mental illness and meeting their needs and those of their carer's and community throughout all phases of care.
- To engage actively and respectfully in reducing barriers to access, providing culturally secure systems of care, and improving social and emotional wellbeing.
- Recognise the potential value of traditional healing practices in the treatment of mental health and social and emotional problems; understand the mental health implications of the history of contact between Aboriginal communities and Australia's mainstream society; and acknowledge that understanding of mental health within Aboriginal communities involve a holistic construct of social, emotional, cultural and spiritual wellbeing.

In addition, mental health services should provide consumers, families and communities with access to services that include the following:

- working collaboratively and in partnership with consumers, families and communities to improve the safety and quality of care provided;
- practicing in accordance with the National Framework for recovery-oriented mental health services;
- having regard to the [Charter of Mental Health Care Principles](#);
- providing trauma-informed care and practice in a strengths-based framework.

Mental Health inpatient services provide assessment, care and treatment for people experiencing acute symptoms and/or behavioural change due to mental illness or psychiatric crises that cannot be managed safely in the community.

Patients accessing mental health inpatient treatment do so on the basis that care, and treatment is provided in the least restrictive manner possible consistent with the *Mental Health Act 2014* (WA) and with regard for safety of patients, staff, carers, visitors and the general public.

All North Metropolitan Health Service Mental Health (NMHS MH) clinicians actively plan as early as possible for the discharge of patients from inpatient care to facilitate effective and timely



discharge as per the National Standards for Mental Health Services and requirements of the *Mental Health Act (MHA) 2014*.

All NMHS MH Services are to develop local protocols, procedures, and tools such as checklists to inform and guide their staff to apply this policy.

3. Risk

Non-compliance with this policy will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☐	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☒	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☒
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

4. Definitions

Consumer	A Consumer is a person with lived experience of a mental health condition who is accessing or has previously accessed a mental health service.
Collaborative care	A more holistic and personalised approach to each person allows them to be partners in their own care, to the extent they desire. Collaborative care plans should be meaningful to the person using their own words and phrases, empowering consumers to take ownership in their own care.
Collaborative care planning	Collaborative care planning prioritises the preferences and values of the person with lived experience of mental health issues, their families, carers and kinship groups and are developed collaboratively with the person and others.
Diversity	Diversity is inclusive of but not limited to the diversity among people with respect to culture, religion, spirituality, disability, power, status, gender and sexual identity and socioeconomic status
Mental State	Mental state comprises mood, behaviour, orientation, judgement, memory, problem solving ability and contact with reality.
Mental State Deterioration	Mental Deterioration is identified based on clinical observation and interviewing and is defined as a <i>“Change in a person’s perception, cognitive function or mood which negatively influences their capacity to function as they would typically choose.”</i>



Peer Worker	Peer workers includes all workers in mainstream or alternative mental health services or initiatives who are employed to openly identify and use their lived experience of mental illness and recovery as part of their work
Recovery	Recovery is defined as “ <i>being able to create and live a meaningful and contributing life in a community of choice with or without the presence of mental health issues</i> ”.
Therapeutic engagement	Therapeutic engagement involves healthcare professionals spending quality time with patients and aims to empower the patient to actively participate in their care. The consumer experiences their interaction with the therapist as one that is safe, compassionate and non-judgemental.
Trauma Informed	Trauma-informed’ services are aware of and sensitive to the dynamics of trauma as distinct from directly treating trauma per se and recognise that consumers are more likely to be re-traumatised when they encounter experiences that trigger memories of past trauma.
ARDT	Admission Readmission Discharge and Transfer
SSCD	State-wide Service Clinical Documentation
CRAM	Clinical Risk Assessment and Management Policy
NaCS	Notification and Clinical Summaries

5. Principles

To facilitate patients’ timely access to admission, care and planned discharge all inpatient services will implement:

- Involvement of the carer, close family member or other personal support person, especially in life-threatening situations. Patient confidentiality should not be used as a reason for not communicating with carers in these situations (see part 9 “Notifiable Events” of the MHA).
- The identification of an allocated Case Manager (community) or CNS (inpatient) or other delegated person responsible for coordinating care and informing the patient and carer, close family member or other support person (if relevant) about any changes in the care management, as per the [NMHS MH Case Management Procedure](#) and the [NMHS MH Role of Allocated Nurse In Inpatient Services Procedure](#).
- The review of any current care plans from a referring service, e.g. a community mental health service (CMHS), to assist with continuity of care.
- A comprehensive care plan, which can be formulated by any member of the multidisciplinary care team with approval by Consultant Psychiatrist or Psychiatrist Registrar, and with the



involvement of the patient and carer, close family member or other personal support person, where possible. The patient is to co-sign their care plan and copies are to be provided to both the patient and nominated carer, close family member or other personal support person.

- A comprehensive risk assessment in line with best practice guidelines, as indicated in the [NMHS MH Clinical Risk Assessment and Management Policy](#). Assessments must consider environmental risks to ensure the patient is cared for in an appropriate clinical setting.
- National Outcome and Casemix Collection (NOCC) outcome measures including the Health of the Nation Outcome Scales (HoNOS); risk assessments inclusive of mental and physical health (including oral health) checks, infection control risks, substance use, and falls/pressure injury risks for at-risk patients; reconciliation of prescribed medications as per the [NMHS MH Medication Prescribing and Administration Policy](#) for Inpatients MHS.
- Completion of the relevant validated assessment tools, the Statewide Standardised Clinical Documentation (SSCD) suite and relevant PSOLIS forms ([SSCD](#))

6. Admission procedures

Access, triage and admission procedures must include the following;

- Defined pathways for admission and appropriate service allocation, e.g. emergency admission (requires admission within 24 hours) or elective admission (admission may be delayed for 24 hours or longer).
- Assessment of risk as indicated in the [NMHS MH Clinical Risk Assessment and Management Policy](#), urgency, distress, dysfunction and disability.
- Minimise delays and duplication in assessment, treatment, care and recovery planning, care delivery and discharge planning.
- Access to appropriate and safe environments conducive to wellness and recovery
- Engage patients nominated other (s) in their care planning and treatment decisions
- Document other nominated carer, close family member or other personal support person's name and contact details in WebPAS, Psychiatric Services On-Line System (PSOLIS) and in the clinical record (in accordance with s296 MHA).
- Provide information regarding entry processes, including criteria for entry to services and information about the services that patients can expect on entering and engaging with mental health services, as indicated in the [National Safety and Quality Healthcare Standards \(NSQHS\) Clinical Governance Standard 1](#).
- Identify persons of concern and patients with identified vulnerabilities (Refer to [MH Person of Concern Protocol](#); [MH Recognising and Responding to Acute Deterioration Policy](#) and [MH Physical Healthcare Management Policy](#)).



6.1 Patients, carers, close family members and personal support persons must be provided with:

- The patient's rights and responsibilities documentation and an explanation by the Consultant Psychiatrist (see the [NMHS MH Rights of Carers and Other Support Persons Policy](#) and s. 246 (b) and s. 245 of the MHA) in a way that is understandable. This must be done as soon as is practicable after admission to the service and at regular intervals throughout the patient's care.
- A verbal explanation of the patient's legal status (voluntary or involuntary) and associated rights as per the [National Charter of Healthcare Rights](#) and the MHA, and about the treatment to be provided to the patient.
- A list of 'out of hours' contact numbers and other support services information, including that of emergency services i.e. the Mental Health Emergency Response Line (MHERL).
- Information about advocacy services, to assist the patient in navigating the system, developing their care plan, and supporting the patient if at risk of not understanding their rights as a patient and citizen.
- Culturally appropriate services.
- Information about referral processes to appropriate specialist clinicians (e.g. Podiatrist, diabetes educator) where indicated.
- Information about in-reach services for mental health patients admitted to a general hospital for treatment.
- The expected length of time the patient will be in hospital (Projected Discharge Date (PDD)) and the admission diagnosis.

6.2 The nominated carer, close family member and personal support person must also be provided with:

- Information about relevant training and services to assist them in understanding the patient's condition, signs and triggers of relapse, and services for advice and support, particularly for times of crises.
- The name and contact details of the allocated Case Manager or Clinical Nurse Specialist (CNS).

7. Discharge procedures

- Discharge planning must be a collaborative process involving the patient, members of the Multi-disciplinary team, nominated persons, carer, close family member or personal support person and/ or guardian/s. Refer to MH [Discharge Planning- For Inpatients](#)
- The authority to discharge a patient from hospital will follow a consultant/team decision process, and this process must be:
- Discussed and completed in collaboration with the multidisciplinary team, the patient, family and carers



- Verbal contact with the relevant triage officer of the receiving community mental health service or other receiving health care provider.
- Clearly documented in medical record and NaCS (discharge summary) which includes recommendations for follow up in the community setting.
- All discharges under the Mental health Act WA 2014 (MHA) Community Treatment Order (CTO) requires verbal consultation between associated Consultant Psychiatrists.
- Utilising standard documentation for service provision, including model of care, patient risk assessment and risk management.
- Compliance with the SSCD electronic information system is mandatory. Completion of the relevant validated assessment tools, the Statewide Standardised Clinical Documentation (SSCD) suite and relevant PSOLIS forms including transfer of information for discharge and timely completion of discharge summaries.
- Risks must be assessed utilising relevant State-wide Standardised Clinical Documentation plus any other endorsed risk assessment tools relevant to the ongoing care of the consumer as per CRAM policy and ARDT guidelines
- Every consumer must have a Management Plan and be given a copy of it. The Management Plan must accompany the consumer between community and other treatment settings and be communicated to new clinicians and relevant others at the time of transition. This ensures that the care “passport” maintains continuity of treatment.
- The family/carer are involved in the discharge planning process and in the development of the discharge plan, as appropriate. The consumer Treatment, Support and Discharge Plan needs to be accurately communicated over time.
- The assessment, care planning and decision to refer a consumer from one public mental health service to another should be a seamless transition and the consumer should not experience further assessments as a barrier to entry.
- All incidents are recorded in PSOLIS and Medical Records are updated as soon as possible (Refer to the [NMHS Clinical Documentation Policy](#) and the [MH Clinical Record Documentation Procedure](#) in accordance with [SSCD](#) requirements).
- Clinical handover is an explicit transfer of information supporting the transfer of clinical accountability and responsibility between healthcare professionals to enable continuity of patient-focussed, safe and high-quality care via a number of formats
- Seven day follow up post discharge is to be completed.

8. Clinical Handover

Clinicians must use the iSoBAR clinical handover process when communicating with member of the multidisciplinary, intake and referral services and when engaging in admission and discharge planning in accordance with the [DoH Clinical Handover Policy and Guidelines](#).

Transfer of care documentation is completed, and iSoBAR clinical handover process followed in accordance with the requirements of the [DOH Clinical Handover Policy and Guidelines](#), the



[NMHS Clinical Documentation Policy](#), the [MH Clinical Record Documentation Procedure](#) and [SSCD](#) requirements.

9. Documentation

Mental health care is especially dependent on data gathering and accurate clinical documentation. Assessment of physical and mental health concerns, diagnosis, and risk identification and management require detailed information often obtained from multiple sources requiring organisation and formulation.

Completion of clinical documentation on PSOLIS and in the Medical Record is in accordance with [SSCD](#) requirements, the [NMHS Clinical Documentation Policy](#) and the [MH Clinical Record Documentation Procedure](#) with the recording of the consumer, nominated carer, close family member or other personal support person's name and contact details in WebPAS, Psychiatric Services On-Line System (PSOLIS) and in the medical record.

Management planning and review is undertaken by a Consultant Psychiatrist led multidisciplinary team of clinicians involved in care provision and is documented in the Medical Record. Management Plans are discussed with the patient (and relevant others with the consent of the patient) to ensure that the patient, advocate, family, and carers are actively involved in care planning, discharge planning and review. The management plan is updated in PSOLIS (signed by the patient) with a copy placed in the Medical Record (Refer to [SSCD](#) requirements).

10. Compliance and Evaluation

NMHS MH Service will monitor performance with the following performance indicators:

- 0% of clinical incidents as a result of admission/entry processes
- 100% of patients to be involved in the development of their management plan
- 100% of nominated carers, close family members or other support persons to be involved in the development of the patient's management plan (with the consent of the patient)
- 100% of nominated carer, close family member and/or other support person's contact details to be entered into Topas/PSOLIS.
- 100% of patients to receive their patient rights and responsibilities within 24 hours of admission (unless exceptional circumstances prevent this from occurring).
- 100% of patients have a comprehensive risk assessment and management plan when admitted to a service.
- 100% of patients will have a discharge plan completed on discharge that includes information regarding risk assessment, management plan, medication reconciliation and follow up arrangements.
- 100% of patients will be contacted within 7 days of their discharge from inpatient services.

Related internal policies, procedures and guidelines

[NMHS MH Clinical Risk Assessment and Management Policy](#)

[NMHS MH Management of Clinical Alerts Policy](#)

[NMHS Clinical Documentation Policy](#)

[MH Clinical Record Documentation Procedure](#)

[NMHS MH Good Practice Guidelines for Engaging with Families and Carers in Adult Mental Health Services](#)

[MH Discharge Planning for Inpatients Procedure](#)

[Triage to Discharge. Mental Health Framework for State-wide Standardized Clinical Documentation \(SSCD\)](#)

[DOH Clinical Handover Policy](#)

[DoH Clinical Handover Guidelines](#)

References

[National Standards for Mental Health Services 2010](#)

[WA DoMental Health Act 2014 WA](#)

[National Safety Quality Healthcare Standards v2.](#)

[Chief Psychiatrist's Standards for Clinical Care](#)

[Chief Psychiatrist Clinicians' Practice Guide to the Mental Health Act 2014](#)

[DoH Community Mental Health Status Assessments: Role of Mental Health Clinician Policy MP 0099/18 \(June 2021\)](#)

[A National Framework for Recovery-oriented mental health services 2013](#)

[Working Together: Aboriginal and Torres Strait Islander Mental Health and Wellbeing Principles and Practice](#)

Useful resources

[Mental Health Coordinating Council \(MHCC\) Recovery Oriented Language Guide](#)

[MHCC A Guide to Culturally Safe Practice in Mental Health](#)



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Policy Contact	Nurse Director MHPHDS				
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Approved by:	Director of Clinical Services MHPHDS			Date:	03/08/2022
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NSQHS Standards Applicable:	☒  Std 1: Clinical Governance ☒  Std 2: Partnering with Consumers ☐  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety		☒  Std 5: Comprehensive Care ☒  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☒  Std 8: Recognising and Responding to Acute Deterioration		
National Standards for Mental Health Services	☒ Std 1: Rights and Responsibilities ☒ Std 2: Safety ☒ Std 3: Consumer and Carer Participation ☐ Std 4: Diversity Responsibility ☐ Std 5: Promotion and Prevention ☒ Std 6: Consumers ☐ Std 7: Carers ☐ Std 8: Governance, leadership and management		☒ Std 9: Integration ☒ Std 10: Delivery of Care ☐ 10.1 Supporting Recovery ☐ 10.2 Access ☐ 10.3 Entry ☐ 10.4 Assessment and Review ☐ 10.5 Treatment and Support ☐ 10.6 Exit and Re-entry		
Chief Psychiatrist's Standards for Clinical Care:	Aboriginal Practice, Assessment, Care Planning, Consumer and Carer Involvement in Individual Care, Physical Health Care of Mental Health Consumers, Risk Assessment and Management, Seclusion and Bodily Restraint Reduction, Transfer of Care				
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Document Version Control			
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07/06/2018	1.2	A/Policy Officer	Review date amended to reflect MHA review; hyperlinks repaired.
09/10/2018	1.3	A/Policy & Audit Officer	Minor amendment, Inclusion of MH and higher order Clinical Care of People who may be suicidal policy documents
16/11/2018	2.0	Clinical Consultant LW OA MH	Scheduled review, update to include links to new CRAM policy, template and hyperlinks
16/01/2019	2.1	A/Policy and Audit Officer	Minor amendment, updated DoH hyperlinks, reformatted document and included NSQHS Standard V2.0
06/02/2021	2.2	Policy and Audit Officer	Minor amendment, link to rescinded DoH CRAM policy removed from text and from references. Link to superseding policy documents added to References other links repaired.
12/08/2022	3.0	Nurse Director MHPHDS	Scheduled review, major amendment, to incorporate information from admission to discharge, changes in documentation and requirements with standards and policy (ARDT)

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative (**ISD Number 1541**).

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